ENSIGN SERVICES

POSTette: The Plan of Care (POC)

Reference: Rehabilitation P&P Policy 220 & 225; Medicare Benefit Manual Chapters 8 & 15

Therapy services must be directly and specifically related to an active written treatment plan that is based upon an initial evaluation performed by a clinician (defined as a therapist not an assistant).

The evaluating clinician will establish a Plan of Care (POC) upon completion of the evaluation process.

Follow Rehabilitation P&P, 220 Evaluation & Plan of Care, for completion time lines.

Plan of Care: The Technical Considerations

- The POC must be established, committed to the electronic record, completed and signed by the clinician.
- The POC must be completed **prior to treatment** being performed or logged by any clinician or assistant. **Note:** The clinician performing the evaluation and establishing the POC may continue the flow from evaluation to treatment on the same date as long as the POC is completed on that date prior to logging any of the treatment session.
- A POC/UPOC may be unlocked after being completed and e-signed by the original completing clinician to make a correction related to a clerical or omission error. Examples: Wrong cert date (start or end or both); wrong diagnostic code(s), but same Medical Condition; Wrong or missing HCPCS/CPT that was intended; wrong frequency.
- The original completing clinician unlocking the POC/UPOC to make the corrections must ensure the following:
    - The corrected POC/UPOC is then e-signed, sent back out to the Physician for signature and date so that the POC/UPOC used for billing has the most up to date corrections
    - The clarification order must match the POC/UPOC
    - The correction MUST BE COMPLETED PRIOR TO TRIPLE CHECK AND/OR PATIENT DISCHARGE FROM SKILLED SERVICE, including Discharge from the facility.
- The POC must be updated (UPOC) by a clinician prior to the expiration of the original certification AND prior to any treatment being provided by any clinician or assistant.
- To avoid not being able to log treatment during the period between the expiration of prior POC and completion of the new UPOC, the UPOC should be completed prior to the current POC's expiration.
- An UPOC is also required any time the initial treatment of the POC is modified significantly (changes the patient's medical condition; new medical diagnoses (not due to coding errors); and any changes to long term goals). Completion of a UPOC for recertification is always required to continue beyond the original cert period.
- Physician/NPP signatures are required within 30 days for all Medicare A, Medicare Advantage and Medicare B payer plans for all POC and UPOCs. Follow company standards for late certs if signatures obtained after 30 days.
- In addition to the POC **a therapy clarification order further supports** the evaluation/ treatment/diagnoses/long term goals, descriptions and plan.

Plan of Care: The Clinical Considerations

The POC summarizes the need for skilled intervention (using the data collected from the evaluation) and establishes services that are reasonable and necessary (the WHY).

Each discipline involved is required to establish their own **plan of care that ties back to the discipline specific evaluations**. The core components of a plan of care include: diagnoses; rehabilitation potential; long and short-term goals; type, amount, duration and frequency of therapy services. In addition, there must be an expectation for improvement of the patient's condition in a reasonable amount of time (e.g. established goals.).

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Frequency

- Frequency refers to the number of times in a week treatment is provided or number of sessions over designated period of time.
- In Net Health Therapy the start of care date (evaluation date) initiates the seven day cycle in which therapy must be delivered to the patient as necessary to meet the physician-prescribed dosage.
- In order to establish organizational consistency, the evaluation encounter (regardless of whether treatment was provided on the same day or not) will count as part of the frequency the first treatment week for most payers.

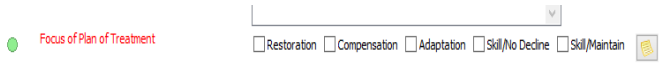
1. The frequency scheduled should match, A. Therapy Clarification Order AND, B. The Therapy Plan of Care.
2. USE THESE TOOLS!! Net Health has supports in place to help leaders with identifying the POC frequency – A. The case manager and the projection screen both show the frequency from the POC. B. The projection screen shows "blue boxes" for the therapy week and the scheduled treatments within those blue boxes should total the frequency. C. The Appointment Book shows the POC Frequency.
3. When a patient has an EXCEPTIONAL need to increase the frequency during a given week, the need for that particular week must be documented in the TEN supporting the added service day, as well as the Progress Report covering that particular week. There must also be an order signed by the physician that supports the additional treatment. If LESS FREQUENT TREATMENT IS PROJECTED than is in the order and POC, an order and documentation supporting the decreased frequency must also be documented in the Progress Report supporting that week. If the delivery of Treatment was less than projected/planned due to an unplanned exception, the reason for a missed treatment must be documented in the service log.
4. When a patient will have an ONGOING need to increase or decrease the frequency for service delivery, a clarification order must be written and a UPOC shall be completed to support the change in the frequency, which is one of the important elements of a POC.
5. The Therapy Orders Weekly Skilled Review Monitoring Tool requires the person completing the monitor to verify frequency delivered to match the order and the POC for each patient. If a frequency exceeds the clarification order/POC and there is no "frequency exception order" written to support the exception, the treatment should be inactivated or changed to "no-bill."

Duration

The duration of the plan should be indicative of the entire course of treatment expected.

Focus of Intervention

Use the date collected from the clinical record, hands on evaluation and any information from the patient/family and/or caregivers, to determine what the focus of therapy will be for each patient: Restoration, Compensation, Adaptation, or Maintenance.



Restoration

- Traditional Rehab
- Good potential for learning or restoring functional abilities
- Optimizes function through dynamic, aggressive rehabilitation treatment
- Treatment integrates multiple functional tasks often involving more than one discipline
- Assesses least restrictive discharge environment
- Training is provided to ensure safe discharge to least restrictive environment
- Resident/family/caregiver education is ongoing

Compensation

- Compensatory strategy training of functional tasks
- Resident may require cueing or assistance to optimize performance
- Identify compensatory techniques that work best for the resident
- Provide training in compensatory strategies to assist the resident in the performance of functional skills

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- Identify and provide training regarding on the selection and use of adaptive equipment for functional and/or leisure tasks
- Family/caregiver education and maintenance programs to assist the resident in carryover of learned compensatory techniques

Adaptation

- Involves caregiver learning to increase safety, quality of life and decrease the burden of care
- Residents may have decreased learning potential
- Identify strategies to prevent further deformities and/or dysfunction
- Focus of treatment is on adaptation of environment to enhance function and caregiver education
- Implements plans for long-term care and support

Maintenance (Skill/No Decline – Skill/Maintain)

- Prevent Further Decline /Maintain the patient's functional capacity
- Improve quality of life
- Decrease caregiving assistance and increase dignity
- Improve or maintain patient safety
- Decrease potential for physical/medical complications

See Functional Maintenance POSTette for additional information.

Goals

A large part of the plan of care includes establishment of long and short-term goals specifically addressing the impairments identified during the evaluation.

Documentation must demonstrate progress toward achievement of goals identified on initial evaluations, and a patient's response to intervention. The reasonableness of time, frequency and duration of intervention must be documented in order to be reimbursed for care.

Goals need to be patient driven, directly tied to functional limitations, related to a functional activity that can be measured, & have a specified time frame.

When establishing patient goals, it is helpful to tie back to the deficits identified on the evaluation and results of standardized testing.

Long-term goals can be developed for the entire course of treatment (evaluation to discharge), rather than just one certification period. These are established by determining what skills will be required in order for a patient to be discharged from therapy services.

Short-term goals are established for an interval (defined as1 -4 weeks, typically do not to exceed 30 days) Goals need to be: measurable, objective, related to functional limitations, have a specified time frame, and be patient driven.

Consider what the medical and functional goals are for the patient with regard to future discharge plans and skill level requirements. The patient's **need** for intervention drives the Plan of Care.

See Goals POSTette for additional information

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